

Stress Urinary Incontinence (Male)

Patient Information · Leaking with effort or activity after prostate treatment

WARWIKI

Stress urinary incontinence (SUI) means leaking urine when something presses on the bladder, such as coughing, standing up or lifting. In men it is almost always caused by a weak urine control muscle after **prostate surgery** or radiation. It often gets better over the first year. If it does not, good treatments are available.

About This Condition

Once the prostate is removed, the **sphincter** (the muscle that holds urine in) does most of the work, and it may be weak. The result is leaking with effort. It can range from a few drops to heavy leakage.

This is different from **urge** leakage, which is a sudden, strong need to go. Urge leakage is treated in other ways. Some men have both.

What Causes It

- **Prostate removal** for cancer, the most common cause
- **Radiation** to the pelvis, or surgery for an enlarged prostate (less common)
- Less often, nerve or bladder problems

Leakage is usually **worst early** and gets better over weeks to months. About 9 in 10 men have good control by 6 months after robotic prostate surgery, and a few more by 1 year. After 1 year, it rarely improves on its own.

Learn the Terms

Stress incontinence

Leaking urine with pressure, such as coughing, standing or lifting.

Sphincter

The muscle that holds urine in until you choose to go.

Prostatectomy

Surgery to remove the prostate.

Pelvic floor

The muscles you train to help control urine.

Pad count

How many pads you use a day, a simple way to measure leakage.

Cystoscopy

A look inside the urethra and bladder with a thin camera.

Male sling

A support placed under the urethra for mild to moderate leakage.

Artificial urinary sphincter

An implant with a cuff and a pump you control.

Will it get better on its own? Often, yes. Most men improve over the first 6 to 12 months. If leakage still bothers you and is not improving, surgery can be considered from about 6 months. It is usually offered at 1 year. Waiting longer is also your choice.

How It's Diagnosed

- Questions about when and how much you leak, often counted as **pads per day**
- A **urine test** and a check of how well you empty
- Before surgery: a **cystoscopy**, and sometimes bladder testing

How It's Treated

These are choices, not steps you must take in order:

- 1 Pelvic floor exercises** (Kegels). They may help you regain control sooner.
- 2 Pads or a condom catheter** while you recover, or long term. A **penile clamp** suits some men: release it at least every 2 hours and never wear it overnight. Do not use one if you have poor feeling in the penis, trouble using your hands, memory problems, or strong urges. Too much pressure can injure the skin and urethra.
- 3 Male sling:** for mild to moderate leakage in men who have not had radiation. Nothing to operate.
- 4 ProACT balloons:** two small balloons adjusted in the office. Not used after radiation. Removal or more surgery is fairly common.
- 5 Artificial urinary sphincter (AUS):** the most reliable choice for any level of leakage, and the usual choice after radiation. More surgery is common over the years.

Each treatment has its own handout.

Living With Male SUI

- Do your pelvic floor exercises every day.
- Use pads or protective underwear while you recover.
- Cut back on evening drinks and caffeine if you leak at night.

Call the office that is treating you if you have:

- A temperature of **100.4 °F (38 °C)** or higher, or burning when you urinate
- Blood in your urine
- Leakage that is getting worse, not better

Go to an emergency department if:

- You cannot urinate for several hours and your bladder feels full or painful
- You cannot reach the office and are worried

Three Things to Remember

- 1.** Leakage after prostate surgery usually improves over the first year.
- 2.** Pelvic floor exercises may help you get control back sooner.
- 3.** If leakage still bothers you, ask about surgery. A sling or AUS can help.